

THE SOMATIC RESET SERIES

The Vagus Nerve Reset

*A 30-Day Protocol to Calm Your Nervous
System*

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Your Nervous System Is Not Broken

Before we talk about fixing anything, let's get one thing straight: nothing is wrong with you. The anxiety, the tension you can't seem to shake, the way small things feel enormous — that's not weakness. That's a nervous system doing exactly what it was designed to do, in a world it wasn't designed for.

The System Nobody Teaches You About

Most of us grow up learning about the heart, the lungs, the digestive system. We get vague warnings about stress being bad. But almost nobody explains the actual mechanism — the specific biological network that determines how calm or how wired you feel on any given day. That network has a name: the autonomic nervous system. And running right through the centre of it is the vagus nerve.

The vagus nerve is the longest cranial nerve in the human body. It starts at the brainstem and winds its way down through your neck, chest, and abdomen,

connecting to your heart, lungs, stomach, intestines, and most of your major organs. It's bidirectional — it carries signals both from your brain to your body and from your body back up to your brain. In fact, roughly 80% of vagal fibres are afferent, meaning they travel upward. Your gut is literally talking to your brain more than your brain talks to your gut.

When researchers talk about the gut-brain connection, the vagus nerve is the highway they're referring to. When you feel something in your chest during a difficult conversation, or notice your stomach clench before a presentation, that's not metaphor. That's the vagus nerve relaying real-time information in both directions.

Polyvagal Theory: A Different Way to Think About Safety

In the 1990s, a neuroscientist named Stephen Porges introduced something called Polyvagal Theory. It reframed the way we understand the nervous system's responses to threat — and it's changed how a lot of trauma therapists, somatic practitioners, and researchers think about anxiety, shutdown, and healing.

Porges proposed that rather than a simple on/off switch between calm and stressed, the autonomic nervous system operates in a hierarchy of three states. The first is the ventral vagal state — this is what most people think

of as being okay. You feel safe, connected, present. Your face is expressive, your voice has warmth, you can think clearly. This is where social engagement lives.

The second state is the sympathetic activation state — fight or flight. When your nervous system perceives threat (real or imagined), it mobilises your body for action. Heart rate climbs. Muscles tense. Digestion slows. Your attention narrows. This is useful if you're running from something dangerous. It becomes a problem when it's triggered by emails.

The third state is dorsal vagal shutdown — freeze or collapse. This is the oldest evolutionary response. When flight or fight seems impossible, the nervous system goes into conservation mode. Energy drops. The world feels flat or unreal. Dissociation, numbness, and that particular flavour of exhaustion where you can't move even though nothing is technically wrong — those are dorsal vagal signatures.

Understanding these three states matters because it explains why trying to think your way out of anxiety often doesn't work. When you're in sympathetic activation or dorsal shutdown, the logical brain is effectively offline. You can't reason yourself down from a triggered state the same way you'd solve a maths problem. The body has to come first.

What Is Vagal Tone, and Why Does It Matter?

Vagal tone refers to the baseline activity of the vagus nerve — specifically the parasympathetic branch that helps your body return to calm after a stress response. High vagal tone means your system is flexible. It can ramp up when needed and come back down efficiently. Low vagal tone means that recovery is slow, or that your baseline is already elevated — as if the system never quite gets the all-clear signal.

Researchers measure vagal tone through something called heart rate variability, or HRV. This is the variation in time between heartbeats. A healthy heart doesn't beat like a metronome — it speeds up slightly on the inhale and slows down on the exhale. That variation is driven by vagal activity. Higher HRV generally indicates better vagal tone, better stress resilience, and better recovery capacity.

The good news — and this is important — vagal tone is not fixed. It's trainable. The practices in this book are not about masking symptoms or managing anxiety in the sense of white-knuckling through it. They're about building the actual capacity of your vagal system so that over time, your baseline shifts. The nervous system that takes three days to calm down after a conflict can, with consistent practice, start to settle in hours. Then

minutes.

Signs Your Vagal Tone Needs Attention

Before starting any protocol, it's worth getting honest about where you actually are. Low vagal tone shows up differently in different people. Some common signs worth noting:

- You get sick frequently, or take a long time to recover when you do
- Digestion is unpredictable — bloating, IBS-type symptoms, constipation that comes and goes
- Sleep is either difficult to achieve or you wake up feeling like you haven't rested
- Social situations feel draining even when they go well
- You startle easily, or stay braced even in objectively safe environments
- Emotional recovery takes longer than it used to — a disagreement stays with you for days
- There's a background hum of anxiety or tension that doesn't have a clear cause
- Your breathing is often shallow, particularly during concentration

None of these is a diagnosis of anything. But they're useful signals that your system is working harder than it

needs to — and that a more grounded baseline is possible.

"The body has a logic of its own. Before you can work with it, you have to stop fighting it."

The Science Without the Jargon

There's a version of this information that gets buried in academic language, and a version that gets so simplified it loses its usefulness. This chapter tries to land somewhere in between — enough to actually understand what's happening in your body, not so much that it becomes another source of overwhelm.

Two Systems, One Body

The autonomic nervous system has two main branches. The sympathetic branch is your accelerator — it ramps things up, prepares you for effort, releases adrenaline and cortisol, diverts blood to muscles. The parasympathetic branch is broadly your brake — it slows things down, promotes digestion and repair, lowers heart rate, signals that the danger has passed.

The vagus nerve is the primary nerve of the parasympathetic branch. When it's functioning well, it acts like a skilled driver who knows when to ease off the accelerator and trust the brake. The problem isn't that we have a sympathetic response — we need it. The

problem is that for a lot of people living with chronic stress, the system gets stuck with the accelerator partially pressed, even at rest. The vagus nerve isn't sending a strong enough signal to the brain that things are okay.

This is what researchers mean when they talk about allostatic load — the cumulative wear from repeated or chronic activation of stress responses. Over months and years, this affects cardiovascular health, immune function, gut health, sleep quality, and emotional regulation. It's not abstract. It has measurable biological consequences.

The Gut-Brain Highway

One of the more remarkable things about the vagus nerve is its relationship with the enteric nervous system — the dense network of neurons embedded in the lining of your gastrointestinal tract. This is sometimes called the second brain, and it contains more neurons than your spinal cord.

The vagus nerve connects these two systems. Gut bacteria produce neurotransmitters — including roughly 90–95% of the body's serotonin — and those signals travel up the vagus nerve to the brain. This is why gut health has such a direct relationship with mood and anxiety. It's not a metaphor. When you have gut

dysbiosis or chronic inflammation in the digestive tract, that information is being sent continuously to your brain. The brain responds accordingly.

Conversely, when you're in chronic stress, the body reduces blood flow to the digestive organs, disrupts the gut microbiome, and impairs the gut lining. Stress causes gut problems; gut problems cause stress. The vagus nerve sits at the centre of this feedback loop, which is why practices that activate it tend to have surprisingly broad effects — better digestion, improved mood, reduced anxiety — that can seem almost too interconnected to believe until you experience it.

Heart Rate Variability: Your Body's Report Card

HRV is worth understanding because it's one of the most reliable windows into vagal function that doesn't require a lab. If you have a fitness tracker, smartwatch, or phone app that measures HRV, the number you see is a reflection of how much variability exists between each heartbeat.

A higher HRV generally means your vagus nerve is actively modulating your heart rate, which reflects good parasympathetic tone. A lower HRV tends to correlate with chronic stress, poor sleep, illness, or nervous system dysregulation. It's not a competition — HRV varies

naturally with age, fitness level, time of day, and individual biology. What matters more than any single number is the trend over time.

During the 30-day protocol in this book, you don't need to track HRV to see results. But if you do have a device that measures it, recording your baseline before and after the protocol can be genuinely useful information. Most people who stick with consistent breathwork and somatic practices see measurable HRV improvements within three to four weeks.

What Healing Actually Looks Like

It's worth setting expectations honestly here, because the way nervous system regulation is sometimes marketed can set people up for confusion. Healing your vagal tone is not a dramatic experience. It doesn't usually feel like a breakthrough. It feels more like a gradual softening — noticing that something didn't bother you as much as it would have, or that you got a full night of sleep without knowing why, or that a conversation that would have left you spinning for hours just... passed.

There will also be periods during the protocol where things feel harder before they feel better. When you begin activating the parasympathetic system more consistently, the body sometimes surfaces things that

were held at bay by constant activation — emotions, fatigue, old tension. This is not a sign that something is wrong. It's the system beginning to discharge what it has been carrying.

Go slowly. This is the most important instruction in this book. The nervous system doesn't respond well to being pushed. It responds to being met.

"Healing isn't always about adding something new. Sometimes it's about finally creating the conditions where the body can do what it always knew how to do."

Week One — Breathe First

The breath is the only autonomic function you can voluntarily control. That's not a small thing. It means you have a direct line to your nervous system, available at any moment, that costs nothing and requires no equipment. Week One is built entirely around this.

Why Breathing Works

When you exhale slowly, you activate the vagus nerve. More specifically, slow exhalations stimulate the baroreceptors in your heart and aorta, which signal the brainstem to increase vagal output — slowing the heart rate and signalling safety to the rest of the system. This is why a long sigh feels relieving. It's not psychological. It's physiological.

The ratio that matters most is the exhale-to-inhale ratio. A longer exhale compared to the inhale consistently increases heart rate variability and parasympathetic tone. Most stressed people breathe in patterns that are the reverse of this — short, shallow inhales and even shorter exhales, often through the mouth, often into the chest rather than the belly.

Changing this doesn't require years of meditation training. It requires about five minutes a day and some patience with the fact that it feels strange at first.

The Physiological Sigh

This is arguably the fastest-acting technique in this book. The physiological sigh is something your body actually does spontaneously during sleep — it's an automatic reset mechanism. Researchers at Stanford, including Andrew Huberman and David Spiegel, published research in 2023 demonstrating that just one to three physiological sighs were more effective at reducing acute stress than five minutes of mindfulness meditation.

Here's how it works: Take a normal inhale through the nose. At the top of that breath, take a second, shorter inhale to fully inflate the lungs — as if topping off. Then release through the mouth in one long, slow exhale. The double inhale reinflates collapsed alveoli in the lungs, which allows the next exhale to offload more carbon dioxide. This shift in CO₂ levels is what triggers the calming response.

Use this any time you feel an acute spike in stress. Before a difficult conversation. After a tense email. In traffic. It works in under thirty seconds and nobody around you needs to know you're doing it.

Box Breathing

Box breathing is a structured practice that became popular partly through its use in military settings, particularly Navy SEAL training, as a tool for managing acute stress in high-pressure situations. Its effectiveness is well-documented and doesn't require you to be in any particular mental state to use it.

The structure is simple: inhale for four counts, hold for four, exhale for four, hold for four. Repeat for four to six cycles. The equal ratio between inhale and exhale creates a balanced autonomic state — not deeply relaxing, but grounded and clear. It's particularly useful before situations that require you to be both calm and alert: presentations, difficult conversations, medical appointments.

For Week One, practice box breathing for five minutes in the morning before you look at your phone. That's the one instruction. Just that, every day for seven days.

Extended Exhale Breathing

This is the deeper relaxation technique. Unlike box breathing, extended exhale breathing is specifically designed to shift you into a parasympathetically dominant state — useful before sleep, after a stressful day, or any time you need to genuinely come down.

The pattern most supported by research is a 4-count inhale through the nose and a 6 to 8-count exhale through the mouth or nose. Some people use the 4-7-8 pattern: inhale for 4, hold for 7, exhale for 8. The hold is optional and some find it uncomfortable — if that's you, simply skip the hold and focus on the longer exhale.

Do this for five minutes in the evening, ideally lying down. If your mind wanders — and it will — that's fine. Return attention to the count. The nervous system doesn't care that you got distracted. It responds to the breath pattern regardless.

Week One Daily Practice

Morning (5 minutes): Box breathing — 4 counts in, 4 hold, 4 out, 4 hold. Six cycles before screens.

Anytime: Physiological sigh when you notice tension, stress, or irritability.

Evening (5 minutes): Extended exhale breathing — 4 counts in, 6–8 counts out. Lying down if possible.

That's the whole week. Don't add anything else yet. The temptation with a new protocol is to do everything at once. Resist it. The point of Week One is to build a consistent relationship with your breath — to make these two practices as automatic as brushing your teeth before you add anything on top of them.

*"You don't have to feel calm to begin. You just
have to begin."*

Week Two — Bringing In the Body

The breath got you started. Now we use the body more directly. The vagus nerve connects to every major organ, and there are several body-based practices that stimulate it in ways breathing alone can't quite reach — particularly for people who carry a lot of tension in specific places.

Cold Water and the Dive Reflex

Splashing cold water on your face, or submerging your face in a bowl of cold water, triggers what's called the mammalian dive reflex. This is an ancient autonomic response that slows heart rate and shifts blood flow toward the vital organs — a parasympathetic effect that's almost instantaneous.

You don't need an ice bath. A cold shower or even thirty seconds of cold water on the face and back of the neck produces a measurable vagal response. The cold water on the face specifically stimulates the trigeminal nerve, which has direct connections to vagal circuits.

Start with thirty seconds of cold water on your face in the morning after your box breathing. If you want to

progress, end your shower with sixty seconds of cold water on the back of the neck and upper back. Most people find this surprisingly pleasant after a few days — the acute discomfort passes quickly and the aftereffect is a particular kind of alertness and calm that's quite different from caffeine.

Humming, Singing, and Gargling

These three practices sound almost too simple to be worth including in a serious protocol. They are, in fact, among the most direct vagal stimulation techniques available — because the vagus nerve has a branch that innervates the larynx and pharynx. Vibrating these tissues through sound directly activates vagal fibres.

Humming: Close your lips and hum any tone for twenty to thirty seconds. Notice the vibration in your chest and throat. Do this three to five times. The effect is subtle but real — most people feel a slight softening or warming in the chest after thirty seconds of sustained humming.

Gargling: Gargle with water for twenty to thirty seconds, twice a day. The vigorous contraction of the throat muscles during gargling stimulates the vagal branches in the pharynx. Stanley Rosenberg, a craniosacral therapist and author who has written extensively about vagal practices, includes gargling as one of the most underrated daily tools for nervous system regulation.

Singing: Any kind of singing — in the car, in the shower, even mouthing along to music — activates the same vocal pathways. This is part of why singing in groups has been shown to increase HRV and reduce cortisol. It's not about musical talent. It's about the vibration.

Shaking and Tremoring

This one feels the strangest to most people, but has the most direct precedent in both animal behaviour and somatic therapy research. When animals in the wild are chased by a predator and escape, they shake. They literally shiver and tremble for several minutes after the threat has passed. This shaking is how the nervous system discharges the mobilised stress energy that was summoned for survival.

Humans suppress this response. We've been socialised out of trembling, which means the discharge mechanism doesn't complete and the mobilised energy stays in the body as chronic tension.

Tension and Trauma Releasing Exercises, developed by David Berzeli, are a systematic approach to deliberately inducing therapeutic tremoring. The exercises tire out the hip flexors and legs in ways that naturally trigger the tremoring response. But you don't need the full TRE system to access some of this. A simple version: stand with feet slightly wider than hip width, bend the knees

slightly, and allow your legs to shake gently for two to three minutes. This can also be done by lying on your back with knees bent and feet flat, then letting the knees fall slightly outward — the inner thighs will often begin to tremble on their own within a minute or two.

This is not for everyone and if it feels overwhelming, stop. But for people carrying a lot of physical tension or who are stuck in chronic sympathetic activation, this practice can produce a remarkable release that breathwork alone doesn't touch.

Week Two Daily Practice

Morning: Box breathing (5 min) + cold water on face (30–60 seconds).

Mid-morning or afternoon: Humming for 2 minutes, gargling once.

Evening: Extended exhale breathing (5 min). Optional: shaking or tremoring for 3–5 minutes before the breathwork if you're carrying physical tension.

Total daily investment: roughly 15–20 minutes. Spread across the day so it doesn't feel like a block of work.

"The body holds what the mind has been trying not to think about. Give it permission to

let go."

Week Three — Deepening the Practice

By now, you've been at this for two weeks. Some things may have shifted — sleep quality, the speed at which you return to calm after stress, a slight loosening of background tension. Week Three builds on that foundation by adding practices that work at longer timescales: meditation, connection, and sleep.

Meditation for Vagal Tone

There is a substantial body of research linking meditation practice to increased HRV and improved vagal tone. A 2015 meta-analysis published in *Psychological Bulletin* found significant effects of mindfulness practice on autonomic nervous system function. More recent research has looked at how even brief, consistent meditation changes the brain's default mode network and strengthens prefrontal cortex regulation over the amygdala — which is the structure that fires the alarm.

The style of meditation matters less than the consistency. Breath-focused meditation, body scan

meditation, loving-kindness practice, and open monitoring all show vagal benefits in different ways. What they share is the fundamental act of returning attention, over and over, without judgment.

For Week Three, add ten minutes of body scan meditation each day. The body scan is particularly effective because it trains interoceptive awareness — the ability to sense what's happening inside your body. This is the same skill that the somatic practices in this book are building. Start at the top of your head and move slowly down through each body region, noticing sensation without trying to change anything. When the mind wanders, return. That's the practice.

Social Connection as Medicine

One of the overlooked aspects of Polyvagal Theory is its emphasis on social connection as a direct nervous system regulator. The ventral vagal system — the one associated with safety and calm — is specifically linked to social engagement. Eye contact, facial expression, tone of voice, physical proximity: these all carry regulatory signals that the nervous system processes at a level beneath conscious thought.

This means that spending time with people who feel safe to you — not just people you tolerate, but people in whose presence you genuinely relax — is a biological

need, not a luxury. And conversely, consistently spending time in social environments where you brace, perform, or manage how you're perceived is a chronic stressor regardless of how functional it looks from the outside.

This isn't an instruction to overhaul your social life. It's an invitation to notice, this week, which interactions leave you feeling more settled and which leave you more activated. And to see if you can, even slightly, weight your time toward the former.

Sleep as a Foundation, Not a Reward

Sleep is when the parasympathetic system does its primary repair work. During slow-wave sleep, cortisol drops to its daily low, inflammatory markers reduce, and the glymphatic system — the brain's waste-clearing mechanism — clears metabolic byproducts, including proteins associated with neurodegeneration. This is not optional maintenance. It's the core process.

Chronic sleep restriction is one of the most powerful suppressors of vagal tone. Even one or two nights of poor sleep measurably reduces HRV and increases inflammatory markers. So for all the breathwork and cold water in this protocol, if you are consistently sleeping six hours or less, you are fighting against your own biology.

Two practices with strong evidence for improving sleep quality through vagal mechanisms: The extended exhale breathing from Week One, done for five to ten minutes in bed, consistently reduces sleep onset time. And avoiding screens for at least thirty minutes before sleep reduces blue light suppression of melatonin — not a vagal mechanism directly, but one that dramatically affects the quality of the sleep you get.

Sleep is not a passive state. It's the most intensive regenerative process your body runs. Treat it accordingly.

Week Three Daily Practice

Morning: Box breathing (5 min) + cold water (30–60 sec).

Midday: 10 minutes body scan meditation.

Afternoon/evening: Humming, gargling, or singing (choose what fits naturally).

Evening: Extended exhale breathing (5–10 min) in bed. No screens for 30 minutes prior.

Throughout the week: Notice one interaction each day that leaves you feeling more regulated. One that leaves you more activated. No judgment needed — just observation.

*"Rest is not laziness. Recovery is not weakness.
The nervous system does its best work when
you stop trying to override it."*

Week Four — Making It Yours

The goal of this final week is not to add more. It's to figure out what actually works for you — specifically, personally, given your life as it actually is — and build that into something sustainable.

What You Should Be Noticing by Now

At four weeks of consistent practice, the changes are usually subtle enough that they're easy to overlook unless you're paying attention. Some people notice it in the absence of things that were previously present — the background anxiety that's quieter, the mornings that don't feel like an assault, the disagreement that settled in an hour instead of a day.

Others notice it in their body — less jaw tension, easier breathing, sleeping through the night more often, digestion that's more predictable. A few people notice it in their HRV if they've been tracking it: a slow upward trend that correlates with the consistency of practice.

What most people don't experience is a dramatic transformation. If you're expecting that, this book probably wasn't quite what you needed — and that's

okay. What you're building here is capacity, not an event.

Building Your Personal Protocol

Over the past three weeks, you've tried several different practices. Some will have resonated and some won't have. That's expected and correct — nervous system regulation is not one-size-fits-all. The practices that work best for you are the ones that your body responds to, not the ones that have the most impressive research behind them.

For your ongoing practice, you need three things: a morning anchor, a daily disruptor, and an evening wind-down. The morning anchor should take no more than ten minutes and should be the first intentional thing you do before screens. The daily disruptor is a brief practice — thirty seconds to three minutes — that you can deploy in the middle of a stressful day. The evening wind-down is what you do in the hour before sleep.

From everything in this book, choose one practice for each slot. Write them down. Commit to them for another thirty days beyond this protocol. The compound effect of nervous system practices is not linear — the benefits accelerate the longer you maintain consistency.

Common Setbacks and What to Do About Them

You will skip days. This is not failure — it's normal. The research on habit formation suggests that what matters is not perfect consistency but how quickly you return after a missed day. Missing one day has negligible effect on the benefits you've built. Missing two weeks and feeling shame about it means you're less likely to restart. So: miss days, return without drama.

Some people find that as their nervous system becomes more regulated, difficult emotions surface that had been held at bay by constant activation. Old grief, anger that doesn't seem to have a clear source, a period of unusual fatigue. This is sometimes called a healing response and is well-documented in somatic therapy literature. If it becomes overwhelming, slow down the practices and consider working with a therapist who has somatic training.

Finally, be cautious about measuring progress against other people's descriptions of their experience. Nervous system healing is deeply individual. Some people feel significant shifts in two weeks. Others are still settling in at three months. Neither is wrong. Your system is responding at the pace that's appropriate for it.

Beyond 30 Days

The practices in this book are not a course you complete and graduate from. They're tools — and like any tools,

they become more useful the more fluently you use them. The physiological sigh doesn't just work in the short term; with repetition, the neural pathways associated with it become more readily accessible, so the calming response is faster and more pronounced.

Consider revisiting this book at the three-month mark with fresh eyes. The chapter that seemed least relevant in week one sometimes becomes the most useful later. Nervous system regulation is a practice that develops over years, and what your system needs will change as circumstances and life do.

There are also deeper modalities worth exploring if you find this work resonates: Somatic Experiencing (developed by Peter Levine), EMDR, craniosacral therapy, and working with a trained Polyvagal-informed therapist. This book is an entry point. What it points toward is a fundamentally different relationship with your own body — one built on attention and trust rather than management and override.

*"You were never meant to hold it all together
all the time. The nervous system was designed
to move through things, not carry them
forever."*

When to Seek Professional Help

Self-directed practices like the ones in this book have real value. They also have real limits. Knowing the difference is important.

The Limits of Self-Practice

The practices in this book are appropriate for general stress, lifestyle-related anxiety, mild-to-moderate burnout, and anyone who wants to build better nervous system resilience as a foundation. They are not designed to address complex trauma, PTSD, dissociative disorders, severe anxiety or depression, or any condition that requires professional clinical support.

If you have a history of significant trauma — particularly developmental trauma or abuse — somatic practices can sometimes be activating in ways that require skilled support to navigate. Going slowly and working with a professional is not a sign that you can't do this work. It's the responsible way to approach it.

Who Can Help

Somatic Experiencing practitioners are trained specifically in body-based trauma resolution using Peter Levine's framework. They work slowly, track nervous system responses in real time, and are skilled at helping clients stay within what's called the window of tolerance — the range of activation that's workable without becoming overwhelming.

EMDR therapists work with the nervous system's processing of difficult memories and experiences, using bilateral stimulation to facilitate resolution. It has strong evidence for PTSD and is increasingly used for a wider range of anxiety and trauma presentations.

Polyvagal-informed therapists may have a range of primary training backgrounds but understand autonomic nervous system responses and work within that framework. The irenics directory (irenics.com) and the USABP (United States Association for Body Psychotherapy) are useful resources for finding practitioners.

Your primary care physician should also be part of the picture if any of your symptoms are unexplained, severe, or significantly impacting daily function. Thyroid conditions, autoimmune disorders, cardiovascular issues, and several other medical conditions can present with symptoms that overlap with nervous system dysregulation. Rule out organic causes before

attributing everything to stress.

*"Asking for help is not the last resort. It's often
the most efficient path through."*
